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TYPES OF CANCER

Liver cancer



LIVER CANCER BOOKLET: EVERYTHING YOU NEED TO KNOW

What is liver cancer?

Primary liver cancer is a malignant tumour that begins in the liver. There are different types:

hepatocellular carcinoma (HCC) or hepatoma, is the most common type of primary liver cancer and it starts in the main cell type in the liver, the hepatocytes

cholangiocarcinoma, or bile duct cancer, starts in the cells lining the bile ducts (which connect the liver to the bowel and gall bladder)

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Understood



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[Secondary cancer in the liver](#) is cancer that started in another part of the body but has spread to the liver. If you have secondary cancer in the liver, it may be useful to read information about the primary cancer in conjunction with this information or about [cancer of unknown primary](#).

A [secondary cancer](#) is named after the primary site where it began, for example bowel cancer with liver secondaries. In this information, we use the term "secondary cancer in the liver" to refer to any cancer type that has spread to the liver.

It is estimated that 3,208 people were diagnosed with liver cancer in 2024. The average age at diagnosis is 69 years old.

Liver cancer signs and symptoms

Liver cancer symptoms are more likely to appear as the cancer grows or becomes advanced.

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pain in the abdomen

swelling of the abdomen due to a build-up of fluid (ascites)

pain in the right shoulder

appetite loss and feeling sick

weight loss

yellowing of the skin and eyes (jaundice)

pale bowel motions

fever

Causes of liver cancer

Hepatitis B or C viruses which result in long term (chronic) infection can cause liver cancer. This is the biggest known risk factor for primary liver cancer in Australia.

There are other factors that can increase the risk of developing liver cancer, such as:

fatty liver disease or genetic disorders including haemochromatosis, or alpha 1-antitrypsin deficiency

type 2 diabetes

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obesity

smoking tobacco

exposure to certain chemicals

Diagnosis of liver cancer

Tests to diagnose both liver cancer and secondary cancer of the liver include:

Blood tests

Blood tests can check how well the liver is working; and how well your blood clots; as well as checking certain chemicals known as tumour markers; whether you have hepatitis B or C; and any genetic problems.

Ultrasound

The most commonly used method to look for primary liver cancer, an ultrasound produces pictures of the organ, and can show the size and location of abnormal tissue in your liver.

CT scan

[CT scans](#) produce three-dimensional pictures of several organs at the same time, and can help doctors plan the [surgery](#). It can also see if the cancer has spread.

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An [MRI](#) scan produces detailed cross-sectional pictures of the body, and can show the extent of a tumor and whether it is affecting the main blood vessels around the liver.

PET-CT scan

More commonly used for secondary cancers in the liver, [PET](#)-CT scans produce three-dimensional colour images that show where any cancers are in the body.

Biopsy

A biopsy is when a small amount of tissue is removed for examination under a microscope.

A biopsy is done by either: fine needle aspiration, which involves a local anaesthetic, and a thin needle which removes the cells; or laparoscopy (keyhole surgery), which is done under general anaesthetic and allows the doctor to look at the liver and surrounding organs, as well as take tissue samples using a laparoscope.

Other tests

If you have secondary cancer in the liver, you may have further tests to find out where the primary cancer started, if this is not already known.

After a diagnosis of liver cancer

After finding out you have liver cancer you may feel shocked, upset, anxious or confused. These are normal responses. For most it will be a difficult time, however

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about your treatment.

Find out more about the best liver cancer care:

What should happen next?

This resource can help guide you and your loved ones after your diagnosis.

[GUIDE TO BEST CARE FOR LIVER CANCER >](#)

Treatment for liver cancer

The most common treatments for primary liver cancer are tumour ablation and chemotherapy delivered directly into the cancer.

The most common treatments for secondary cancer in the liver are chemotherapy or a combination of surgery and chemotherapy.

Surgery

Surgery can involve removing part of the liver, or a liver transplant, where the whole liver is replaced by a donor liver.

Surgery is suitable for a small number of people with liver cancer, and it will depend

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back to normal size over a few months. If there are tumours in both sides of the liver, surgery may be conducted over two stages, to allow the liver to regrow following the first operation.

For some people, it is not possible to remove part of the liver, and they may be considered for a transplant. A number of factors are considered before someone is eligible for a liver transplant, including the person's overall health. In addition, a person should not smoke or take illegal drugs and should have stopped drinking alcohol for at least six months. A liver transplantation may be possible, however, waiting for a suitable donor liver may take a long time. During this time, most people have other treatment to control the cancer.

Tumour ablation

Tumour ablation is used most commonly for small primary liver cancers. It is rarely used for secondary cancer in the liver.

The most common types of tumour ablation treatments use radio waves and microwaves to heat and destroy cancer cells. This treatment is administered by either a needle inserted through the skin (percutaneous ablation) or a surgical cut (ablation with surgery).

Less common types of ablation treatment include alcohol injection, where pure alcohol is inserted into the tumour to destroy cancer cells; and cryotherapy, which is surgery that freezes and kills cancer cells.

Chemotherapy

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Chemotherapy can be given systemically, via tablets or intravenously (through a drip), which means it goes throughout the whole body. This may be given following other treatment, such as surgery or cryotherapy, to get rid of any remaining cancer cells. It is not usually used to treat primary liver cancer unless the cancer has spread to other parts of the body.

Systemic chemotherapy can also be used as palliative treatment to slow down the cancer growth and reduce pain.

Chemotherapy can also be given directly into the tumour, which is called chemoembolisation (or TACE). As it is targeting the tumour directly, stronger drugs can be used. TACE is more commonly used for primary liver cancer.

Biological therapy

A range of treatments that work against the cancer cells by either stopping their growth or function, or by helping the body's immune system destroy them. Used for both primary and secondary cancers in the liver, it can be used in conjunction with, or after other treatments for cancer.

Selective internal radiation therapy (SIRT)

Also known as radioembolisation, this treatment targets liver tumours directly with high doses of internal radiation in tiny beads. It is used for both primary and secondary cancers in the liver when the tumours can't be removed with surgery. It is not available in all areas, so talk to your doctor about availability and costs involved.

Endoscopic stent placement

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Palliative care

In some cases of liver cancer, your medical team may talk to you about [palliative care](#). Palliative care aims to improve your quality of life by alleviating symptoms of cancer.

As well as slowing the spread of liver cancer, palliative treatment can relieve pain and help manage other symptoms. Treatment may include [radiation therapy](#), chemotherapy or other drug therapies.

Treatment Team

Depending on your treatment, your treatment team may consist of a number of different health professionals, such as:

GP (General Practitioner) - looks after your general health and works with your specialists to coordinate treatment.

Hepatobiliary surgeon - specialises in surgery of the liver, pancreas and gall bladder and surrounding organs.

Gastroenterologist - specialises in diseases of the digestive system.

Hepatologist - gastroenterologist who specialises in diseases of the liver.

Medical oncologist - prescribes and coordinates the course of chemotherapy.

Radiologist - interprets diagnostic scans (including CT, MRI and PET scans).

Nuclear medicine specialist - coordinates the delivery of radioactive scans and

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Other allied health professionals – such as social workers, pharmacists, and counsellors

Screening for liver cancer

There is currently no national screening program for liver cancer available in Australia.

Preventing liver cancer

Hepatitis C and B infections are the biggest known risk factors for primary liver cancer in Australia. There are prevention strategies to reduce the spread of hepatitis B and the incidence of primary liver cancer, including vaccinating all at-risk people against the virus, and monitoring the health of people already infected with the virus.

Prognosis of liver cancer

Prognosis means the expected outcome of a disease. An individual's prognosis depends on the type and stage of cancer, as well as their age and general health at

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growth, as well as your age, fitness and medical history.

The prognosis for liver cancer is better when diagnosed in the early stages.

Sources

Understanding Cancer in the Liver, Cancer Council Australia © 2024. Last medical review of source booklet: July 2024. All updated content has been clinically reviewed by A/Prof Simone Strasser, Head of Department and Hepatologist, AW Morrow Gastroenterology and Liver Centre, Royal Prince Alfred Hospital and The University of Sydney, NSW; and A/Prof Siddhartha Baxi, Radiation Oncologist and Medical Director, GenesisCare, Gold Coast, QLD. This edition is based on the previous edition, which was reviewed by the following panel: A/Prof Simone Strasser (see above); A/Prof Siddhartha Baxi (see above); Prof Katherine Clark, Clinical Director of Palliative Care, NSLHD Supportive and Palliative Care Network, Northern Sydney Cancer Centre, Royal North Shore Hospital, NSW; Anne Dowling, Hepatoma Clinical Nurse Consultant and Liver Transplant Coordinator, Austin Health, VIC; A/Prof Koroush Haghighi, Liver, Pancreas and Upper Gastrointestinal Surgeon, Prince of Wales and St Vincent's Hospitals, NSW; Karen Hall, 13 11 20 Consultant, Cancer Council SA; Dr Brett Knowles, Hepato-Pancreato-Biliary and General Surgeon, Royal Melbourne Hospital, Peter MacCallum Cancer Centre and St Vincent's Hospital, VIC; Lina

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